



Longevity Guide: Fix Your Insomnia In 8 Weeks

Evidence-Based and Self-Guided Strategies
to Manage Insomnia and Optimize Sleep

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MEDICAL DISCLAIMER

This guide is intended for informational and educational purposes only and is not a substitute for professional medical advice, diagnosis, or treatment. Always consult a qualified healthcare provider before starting, changing, or discontinuing any supplement, medication, or health regimen.

Some treatments or devices discussed may not be FDA-approved for all sleep disorders.

Individual responses vary, and dosages listed here reflect ranges studied in scientific literature, not personalized medical recommendations.

ABOUT THE AUTHOR



Hello there!

I'm Dr. Sahil Chopra, MD. I'm a quadruple-boarded physician specializing in Internal, Sleep, and Pulmonary/Critical Care Medicine. I completed my Internal Medicine residency at UCLA, Pulmonary/Critical Care fellowship at Loma Linda University, and Sleep Medicine training at Harvard.

We started Empower Sleep with one vision in mind: to provide Harvard-grade sleep care to as many patients as possible. The strategies I adopted in my Sleep Medicine fellowship were deeply rooted in the science and physiology of sleep science. We now apply this science here at Empower Sleep. The goal is to treat all patients as an "n-of-1" (individualized approach). Not a homogeneous, population health-based approach, which is what happens in traditional healthcare. Everyone's sleep is unique to them, as is their sleep disorder(s). When we approach every patient as an individual, we can typically get to the root cause, find a personalized solution, and have stellar clinical outcomes.

At Empower Sleep, we have cared for thousands of patients and analyzed hundreds of thousands of nights of sleep biomarkers. Our innovative care model and success is a by-product of my mentors, my amazing team, our patients, and my family.

Thank you in advance for taking the time to improve your sleep health. I hope you find this guide as a helpful tool to solve your sleep struggles.

It is my wish for all humans to live healthy and fulfilled lives.

Dr. Sahil Chopra, MD



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Introduction



For many of my patients, insomnia is not just about a few bad nights of sleep. It's a relentless cycle of tossing and turning, waking at odd hours, and starting the day already exhausted. I know how discouraging this feels. But I also know that insomnia is not a life sentence.

Over the years, I've learned that insomnia is best approached as a **journey and process**. It's not a quick fix. It requires work by the patient, including patience, consistency, and a willingness to experiment with new behaviors. This guide is my way of walking you through that journey, step-by-step, with the best evidence we have today.

Think of this as a structured, eight-week plan that empowers you to reclaim your sleep.

01 What is Insomnia?

Insomnia isn't defined by the number of hours you sleep. It's about how you function. If you lie awake at night unable to fall asleep, wake repeatedly, or rise far earlier than intended—and these struggles leave you impaired during the day—we call this clinically significant insomnia.

There are **two broad categories** of insomnia:

- **Acute insomnia** occurs during stressful periods—a job transition, exams, or grief—and often resolves once the stressor passes.
- **Chronic insomnia** persists for at least three nights per week over three months or longer.

Chronic insomnia is especially important to address because it doesn't just fade away with time. It reshapes how the brain regulates sleep and wakefulness.

Acute Insomnia



Chronic Insomnia

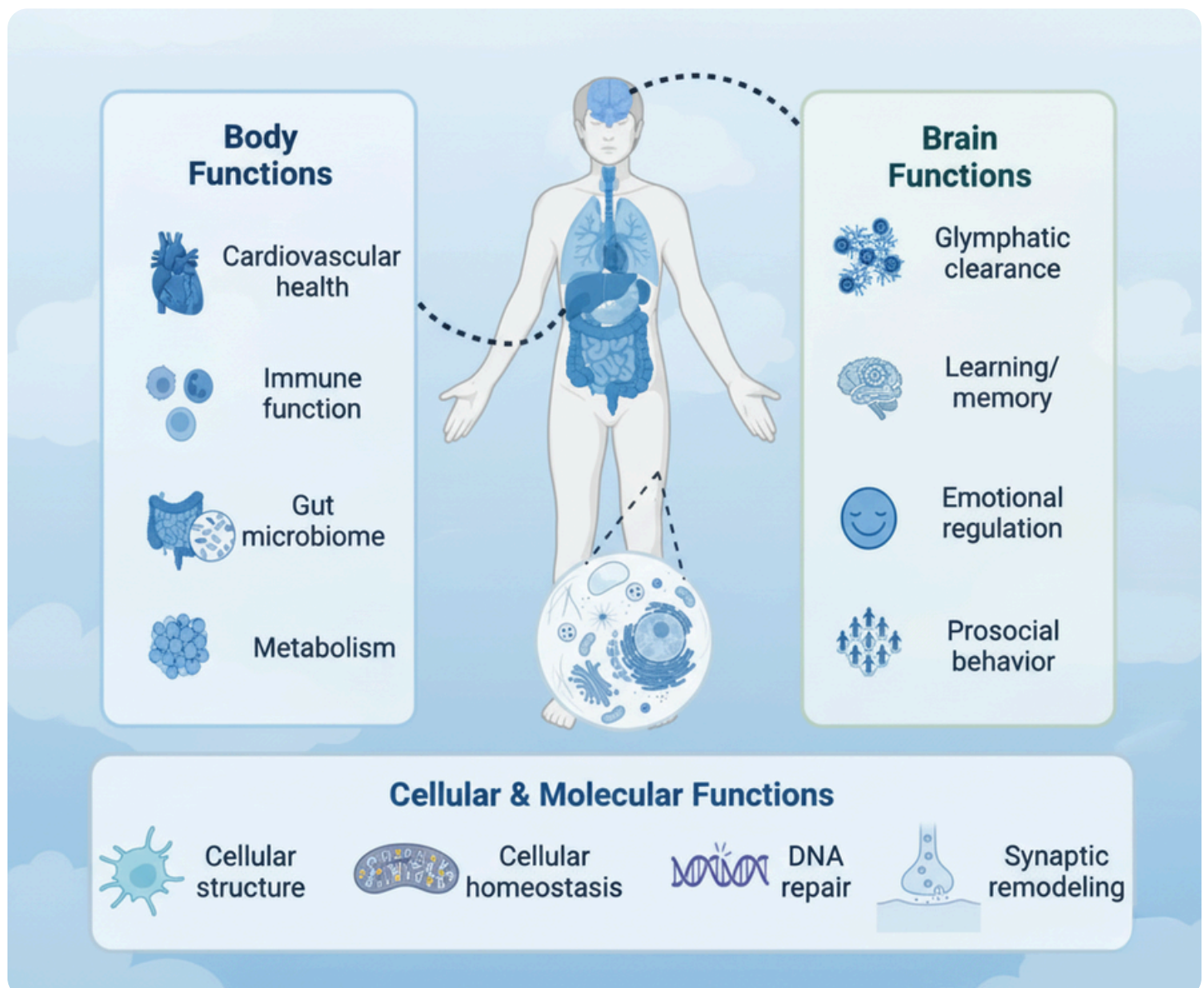


02 Why Insomnia Matters

Some people dismiss insomnia as an annoyance. But the science tells us otherwise. Chronic insomnia increases the risk of depression and anxiety, worsens blood pressure, and raises the risk of heart disease. It interferes with memory, concentration, and performance at work. In some cases, it even weakens the immune system and alters metabolism, contributing to weight gain and insulin resistance.

This is why I take insomnia so seriously in my practice. **Sleep is not a luxury. It's a biological necessity.** If you're struggling with insomnia, you are not simply tired. Your whole system is under strain.

Healthy sleep plays a critical role in every aspect of physiologic functions, including:



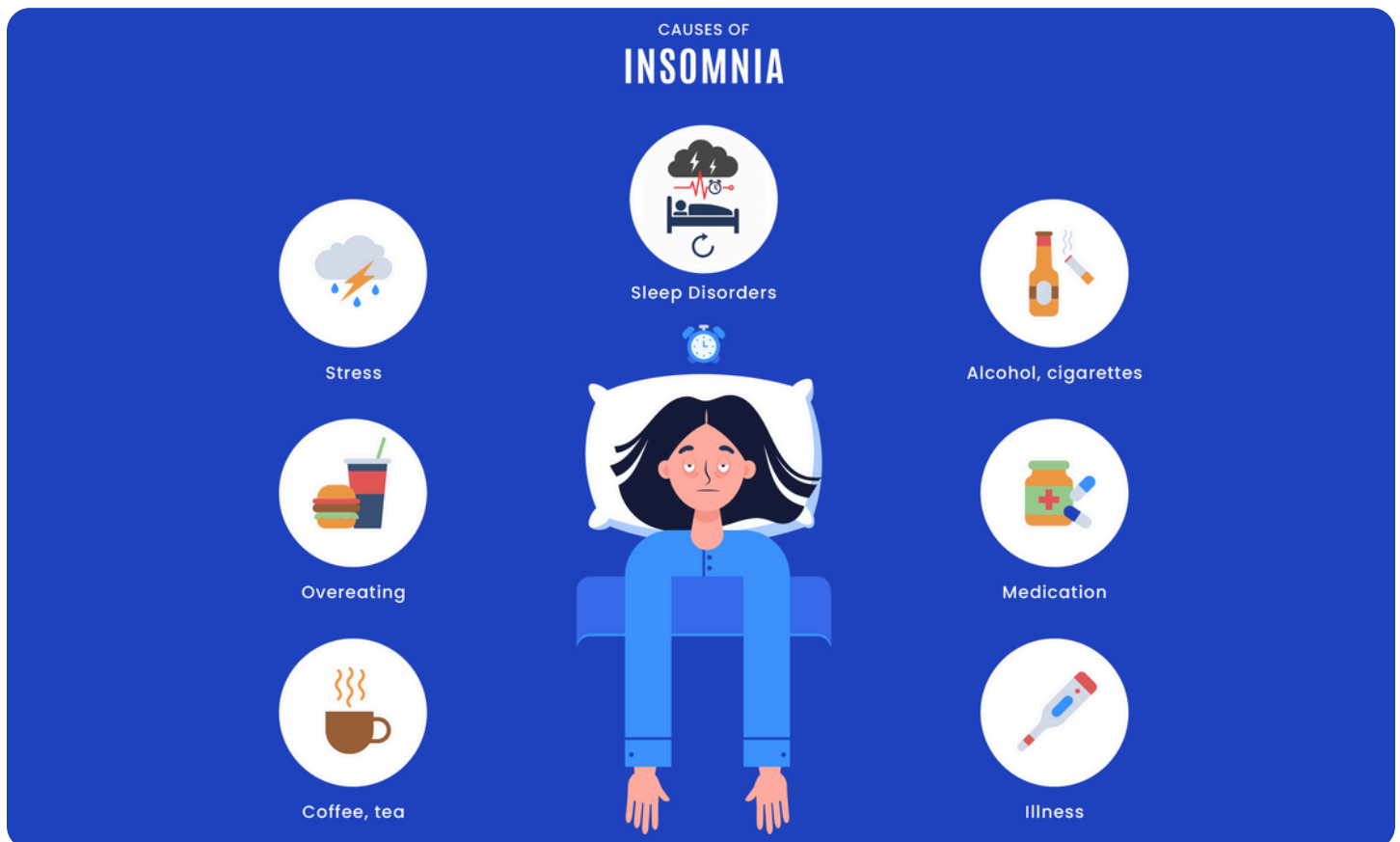
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Common Causes of Insomnia

Patients often ask me: “Why can’t I sleep? What’s broken?” My answer is that insomnia usually arises from a mix of factors rather than one single cause and it becomes an ongoing chronic problem when we develop “maladaptive” behaviors while trying to compensate.

A few things to note:

- Stress and anxiety are among the biggest drivers of insomnia
- Medical conditions like chronic pain, thyroid disorders, reflux, or asthma can also keep the body in a state of hyperarousal, a state of increased physiological and psychological activation
- Medications (e.g. steroids, stimulants, antidepressants, etc.) often worsen sleep
- Hormone and nutrient deficiencies can cause symptoms of insomnia
- Caffeine and alcohol can disrupt normal sleep patterns
- Behavioral/environmental contributors such as irregular schedules, late-night screen exposure, noise, or subtle bedroom light signals the brain to stay alert when it should be winding down
- Other sleep disorders like sleep apnea, Restless Legs Syndrome (RLS), circadian rhythm disorders are common drivers of insomnia



04

THE SCIENCE OF SLEEP-WAKE REGULATION

Healthy sleep-wake patterns are governed by a homeostasis of two systems:

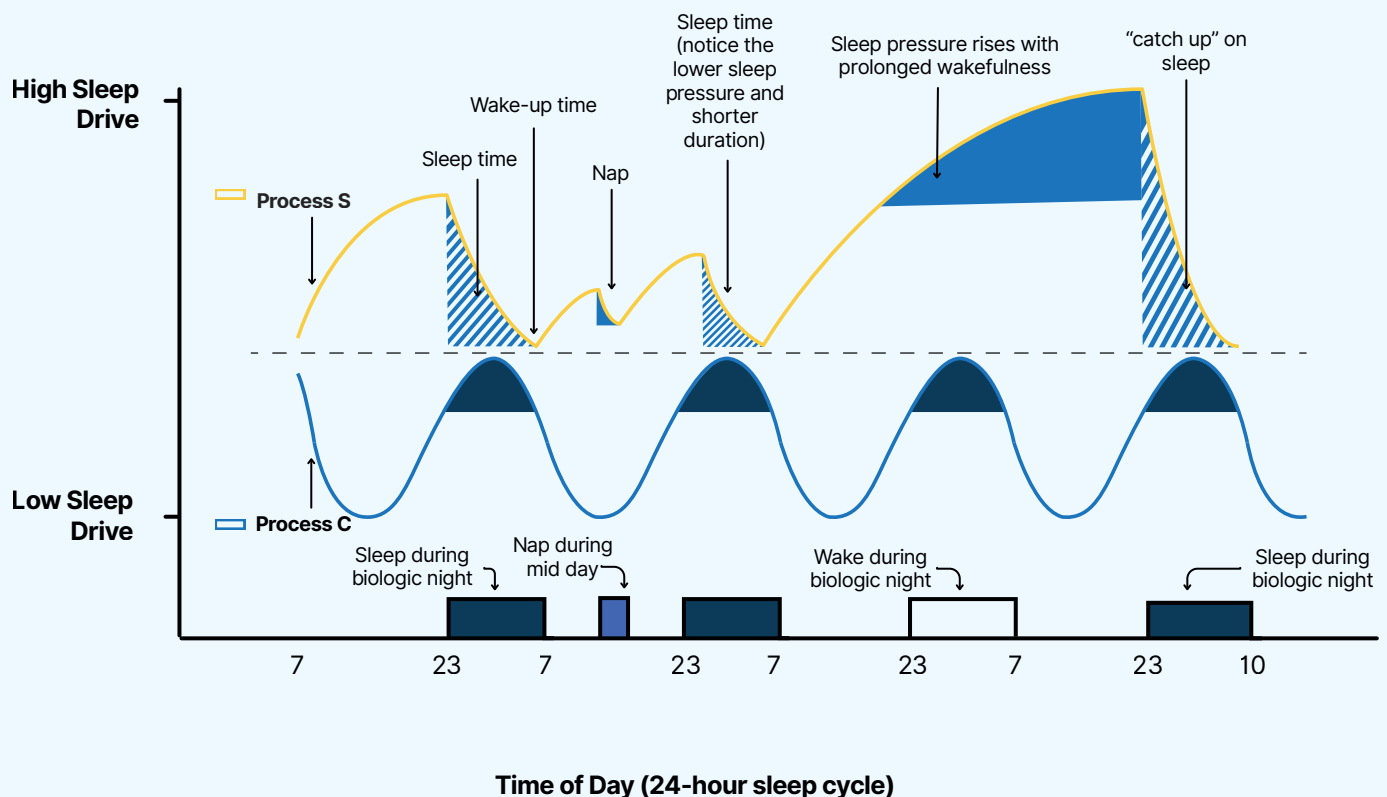
- **Process S (Sleep Drive and Sleep Pressure):** Builds steadily through the day. The longer you're awake, the stronger your drive to sleep, and the “sleepier” you feel
- **Process C (Circadian Rhythm):** Your internal clock, which is regulated by light and darkness. It determines when your body naturally wants to be awake or asleep

Insomnia can take root when these two systems fall out of sync.

The ideal opportunity to “consume” sleep is when both systems align—at the peak of sleep drive (~16 hours after being awake) and during circadian nightfall.

The diagram below shows the relationship of these two processes and how naps and prolonged wakefulness influence our regulation and need for sleep.

Sleep-Wake Cycle: A Two Process Model



05 THE SCIENCE OF SLEEP STAGES

At night, we cycle through different stages of sleep. Each cycle lasts 80-120 minutes. Broadly speaking, there are 2 stages of sleep (NREM and REM). Assuming we sleep 7-9 hours, our sleep architecture should be as follows:

NREM ("Non-Dream Sleep") is 75-80% of total sleep time

NREM is divided into 3 stages (N1, N2, N3). Generally speaking the sum of N1+N2 is referred to as light sleep and N3 is considered deep sleep.

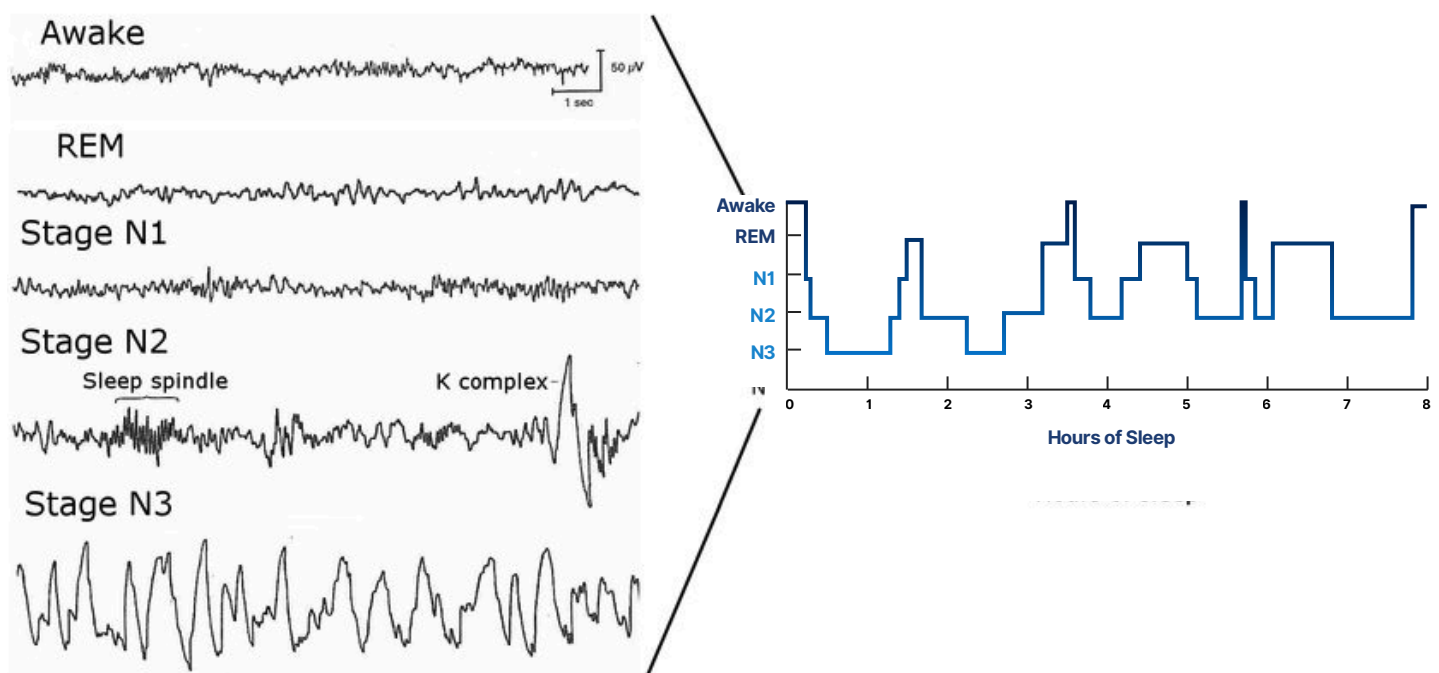
- **Stage N1:** 2-5% of total sleep time, 10-20 minutes, transition from wake to light sleep
- **Stage N2:** 45-55% of total sleep time, 3-4 hours. The beginning of true sleep
- **Stage N3, slow wave or deep sleep:** 20-25% of total sleep time, 1.5 - 2 hours. This should be the most parasympathetic predominant state (rest and digest).

REM ("Dream Sleep"). 20-25% of total sleep time

- Complete loss of muscle tone and rapid movements of eyes. Brain waves resemble being awake except our body is actually paralyzed so we don't act out our dreams

Additional things to note:

1. Not all sleep cycles are the same. The first half of the night is predominately more deep sleep per sleep cycle whereas the latter is more REM sleep per cycle
2. Being awake and REM sleep look similar for a brain wave and autonomic nervous system standpoint
3. Deep or N3 sleep is distinctly different and critical for healthy sleep



06

SLEEP CHANGES OVER A LIFESPAN

To better understand insomnia, it is critical to be aware how our sleep changes over our lifespan.

Assuming you're out of your 20s, it is not biologically possible for you to have the same "sleep health" as you did when you were 21. As you look at the diagram below, you'll notice a few things regarding common sleep metrics:

- **Total Sleep Time (TST) on the 'y' axis:** You'll notice the total "appetite" for sleep drops as we age. This basically means, **we can not sleep as much as we did when we were younger**. I used to have an older Attending in my critical care fellowship that made fun of us for needing more sleep than him. In hindsight, the joke is on him. This is a sign of aging.
- **Slow Wave Sleep (aka SWS, N3, Deep Sleep):** In addition to a steady decline with age, there are 2 accelerated drop offs. Firstly, during your teens going into your 20s and again in your 60's going into your 70s. Deep sleep declines most significantly across lifespan.
- **REM, N1, and N2:** You'll notice they have a similar distribution during our lifespan.
- **Wake After Sleep Onset (WASO):** This metric sounds funny, but basically it refers to the cumulative time you are awake **after** you have first fallen asleep during the night. You will notice this starts to increase after your mid-40s.

Sleep Across a Lifespan

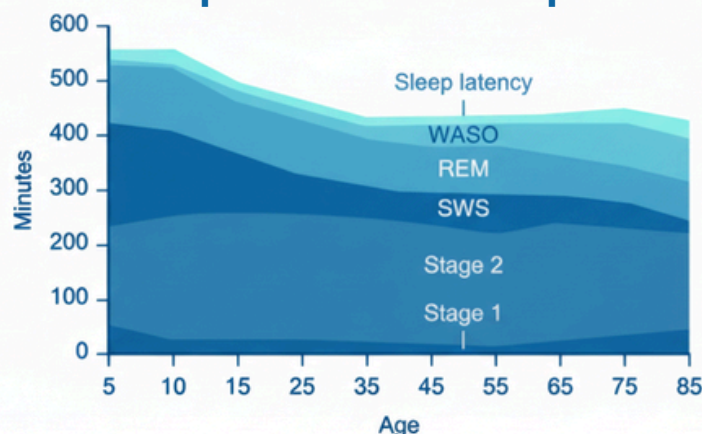


FIGURE 3.5 Changes in Sleep with Age, Sleep time (in minutes), wake time, sleep latency, and sleep stages change with aging. REM, Rapid eye movement; SWS, slow wave sleep; WASO, wake after sleep onset. (From Carskadon MA, Dement WC. Normal human sleep. In: Kryger MH, Roth T, Dement WC, eds. *Principles and Practice in Sleep Medicine*. 6th ed. Philadelphia, PA: Elsevier, 2017:15-24.)

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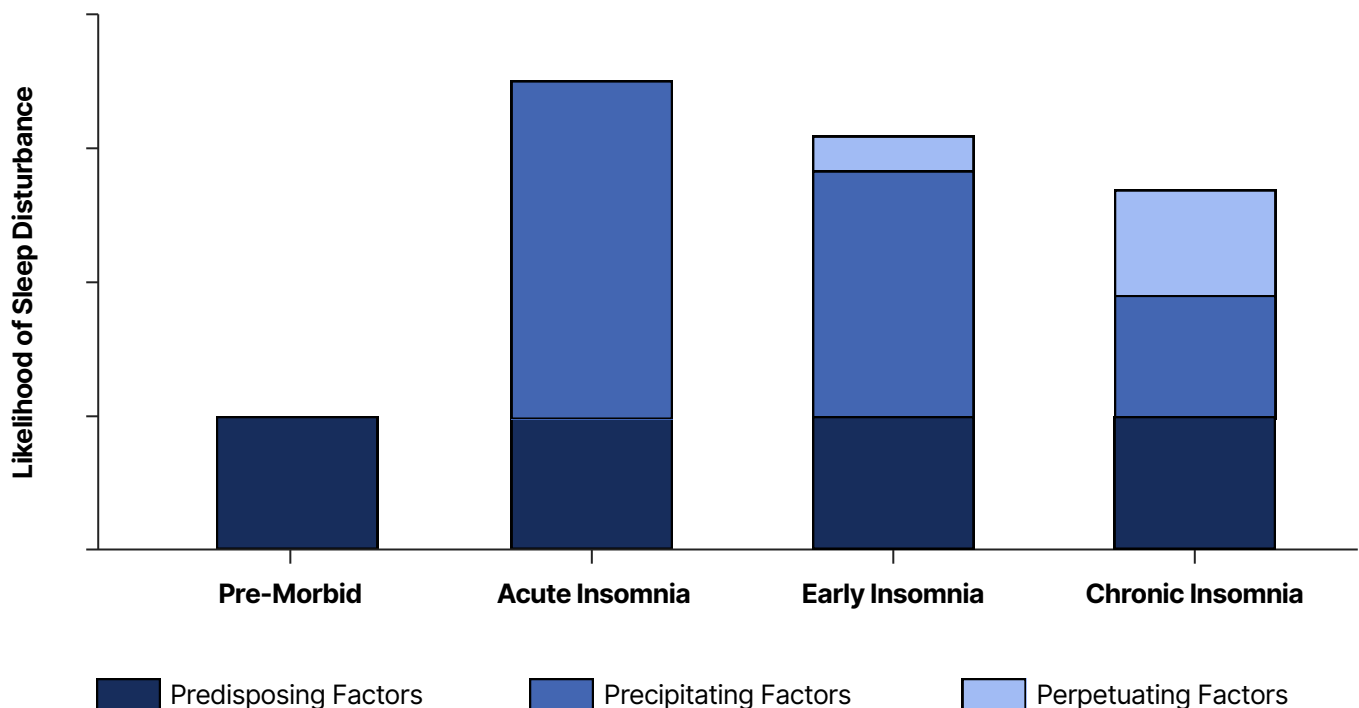
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UNDERSTANDING THE 3P MODEL OF INSOMNIA

One of the most helpful ways I explain insomnia to patients is through the **3P Model**. This framework was developed by Dr. Arthur Spielman. It illustrates how insomnia develops and persists.

- **Predisposing factors** are the underlying traits that make someone more vulnerable to insomnia. These can include being naturally anxious, having a family history of poor sleep, or even being a “light sleeper” since childhood. Think of these as your pre-determined susceptibility to insomnia
- **Precipitating factors** are the triggers and stressful life events that “spark” insomnia. This might be a job loss, a breakup, a medical illness, or even something as common as jet lag. These are the “second hit.”
- **Perpetuating or maladaptive factors** are the habits and thought patterns that keep insomnia going long after the original precipitant is gone. These are typically behaviors that maintain the symptoms of insomnia. For example, going to bed earlier “just in case,” scrolling on your phone while trying to fall asleep, or ruminating about not sleeping. These factors reinforce the problem, which turn short-term insomnia into chronic insomnia

The beauty of this model is that it gives us a **roadmap for treatment**. We can’t change your genetic predisposition. We can’t always avoid life stressors. But we can work on the perpetuating and maladaptive behaviors. That’s exactly what therapies like Cognitive Behavioral Therapy for Insomnia (CBT-I) target. By breaking the cycle of unhelpful sleep habits and thought patterns, we can address the primary cause without developing a dependence on medications.



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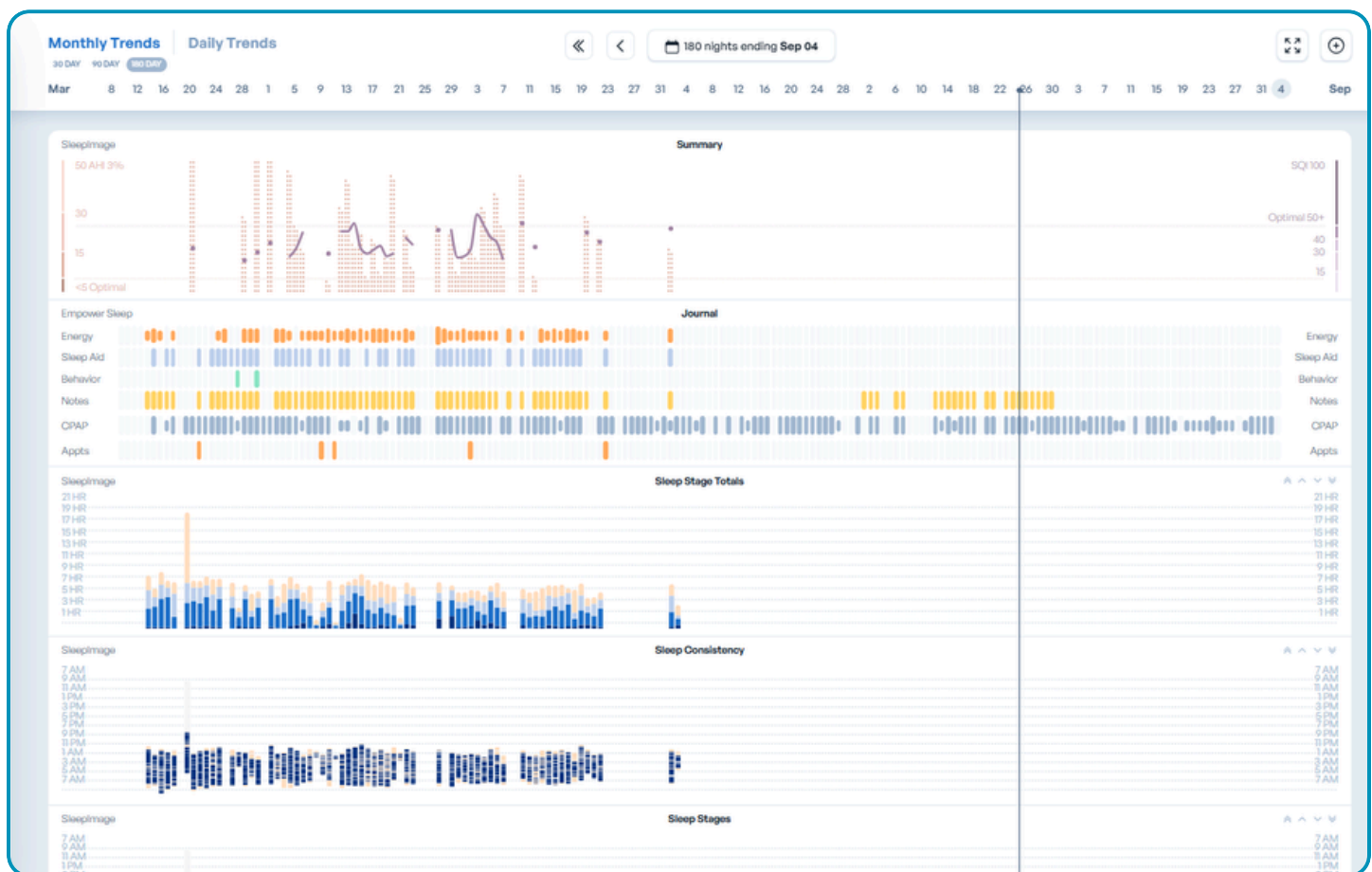
THE ROADMAP: AN 8-WEEK JOURNEY

I've designed this program as an **eight-week self-guided journey**. Each two-week block has specific goals.

Weeks 1–2: Tracking and Awareness

Start by observing. Keep a **sleep diary**, use a **wearable tracker** or your **Empower Sleep** app if you are a member. Take a note of when you go to bed, when you wake up, how rested you feel, and any naps or caffeine use. This isn't about judgment. It's about raising awareness and seeing patterns.

Below is an example of how we track sleep patterns for our members across time, including different biomarkers and vitals that help us better assess sleep health.



Weeks 3–4: Building the Foundation

Once you've identified your patterns, let's restructure them. This is where **stimulus control therapy** comes in: only use your bed for sleep and intimacy, nothing else. If you can't fall asleep within 20 minutes, leave the bedroom and return only when sleepy.

It is critical to create a routine of non-stimulating/relaxation-promoting activities one hour prior to habitual bedtime. Practices include dimming lights, breathing exercises, meditation, etc. Abstaining from wake-promoting activities such as TV/Netflix and answering emails.

During this time, it is also critical to have good circadian cues such as light exposure upon awakening and darkness exposure prior to bedtime.

We also use **Sleep Restriction Therapy (SRT)**. Paradoxically, by reducing your time in bed to match the time you're actually asleep, we train the brain to build stronger sleep pressure. If your subjective or objective sleep is 6 hours, but you spend 8 hours in bed, I would ask you to only spend 6 hours in bed (depending on your symptoms).

Over time, we gradually expand the sleep window as your Sleep Efficiency (SE) improves.

Below is an **example** of an individual who has sleep maintenance insomnia (Total Sleep Time of 6 hours but spends 10 hours in bed from 10pm - 8am). Their Sleep Efficiency is therefore 60%.

Week	Bedtime	Wake-Up Time	Total Time in Bed	Instructions/Guide
Baseline (Pre-SRT)	As per usual	As per usual	Record actual sleep time and time in bed for 1-2 weeks	Track current sleep patterns to establish an average Total Sleep Time (TST) and Sleep Efficiency (SE).
Week 3	12:00 AM	6:30 AM	6.5 hours	Continue adjusting Time In Bed (TIB) by 15-30 minutes only if SE is 85% or higher. If SE drops below 80%, decrease TIB by 15-30 minutes.
Week 4	Adjusted	Adjusted	Adjusted	Continue gradual adjustments until an optimal sleep duration is achieved with good sleep quality. The goal is to consolidate sleep and improve Sleep Efficiency, then slowly expand the sleep window to a comfortable amount
Maintenance	Individualized	Individualized	Individualized	Once optimal sleep is achieved, maintain a consistent sleep schedule. Re-evaluate if sleep problems return.

Weeks 5–6: Cognitive and Behavioral Training

These weeks focus on the mind. Many insomnia patients wrestle with racing thoughts — “What if I don’t sleep tonight? What if tomorrow is ruined?” This is where **cognitive restructuring** helps. We challenge those catastrophic thoughts with more balanced thoughts.

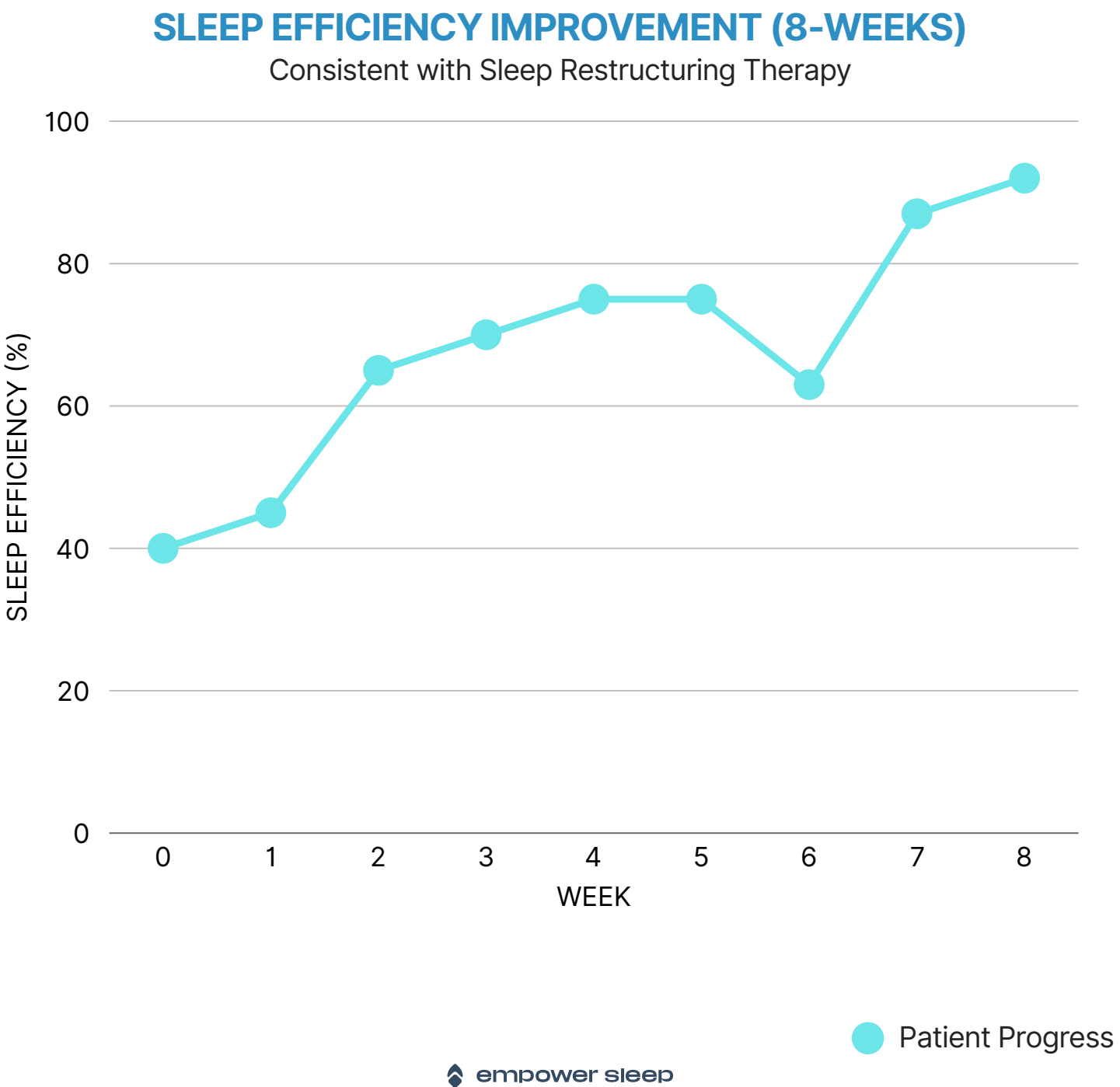
Relaxation practices like progressive muscle relaxation or guided breathing reduce physiological arousal. For some patients, I even recommend writing down worries before bed to “park” them for the night.



Weeks 7–8: Optimization and Long-Term Success

At this stage, we fine-tune. We continue strengthening circadian cues with morning light exposure and consistent wake-up times. We optimize the sleep environment, which should be dark, quiet, and cool. We also troubleshoot lingering barriers: early awakenings, anxiety spikes, or reliance on trackers.

The goal isn't to have perfect sleep every night. No one has that. The goal is to build a resilient system where temporary stressors no longer spiral into chronic insomnia.



09

LIFESTYLE AND MIND-BODY APPROACHES

All humans should participate in some degree of regular physical exercise.

Exercise, especially aerobic activity, has been shown to improve sleep quality.

Mind-body approaches such as meditation, yoga, and breathwork reduce nighttime arousal, making it easier to transition into restorative sleep. Exercise performed earlier in the day (especially outdoors) can enhance circadian alignment, while late-evening high-intensity exercise may delay sleep onset in sensitive individuals.



10

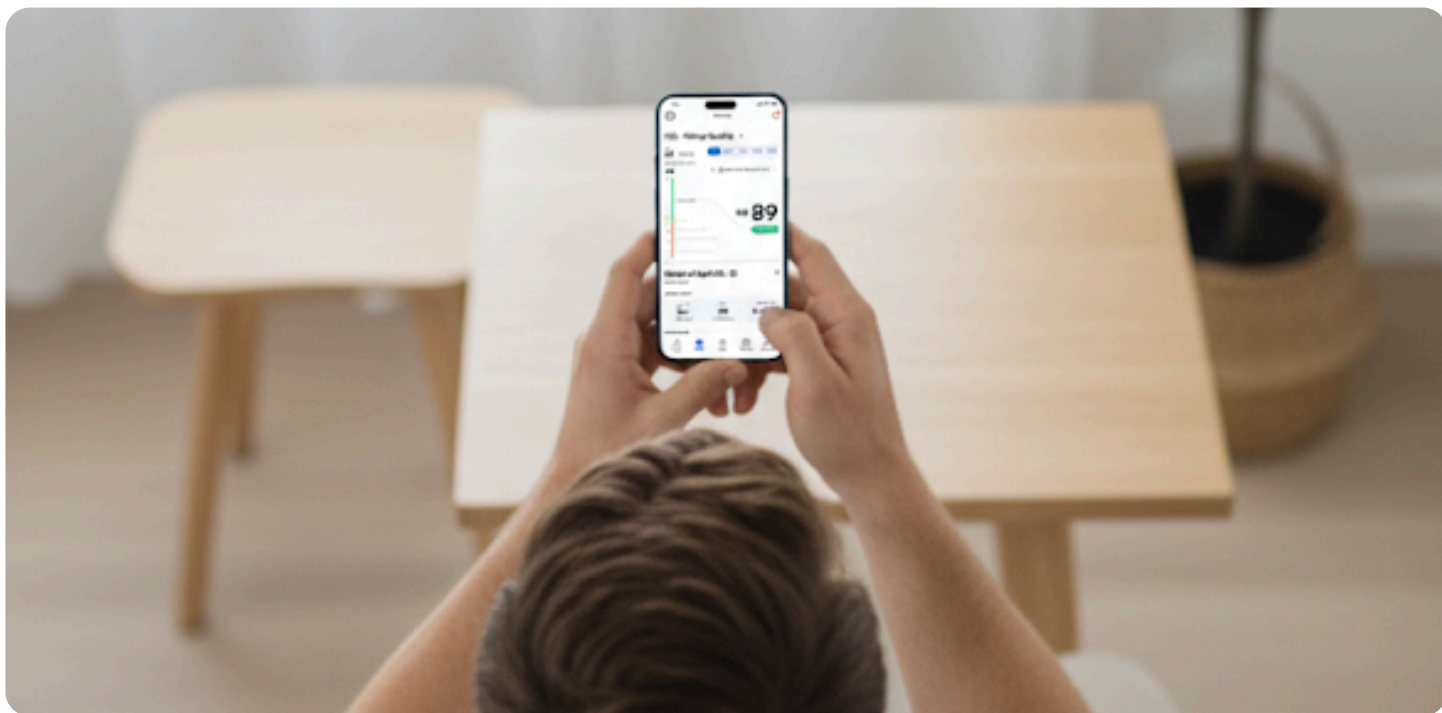
USING WEARABLES FOR SLEEP ASSESSMENT

I often get asked: “Can I use my wearable to track my sleep?” The answer is yes, as long as we are not letting the data drive anxiety, fear, or discouragement. If it makes us more worried, then take a break.

I also often get asked: “can my wearable replace a sleep study?” The answer is no. Wearables don’t measure brain waves and can’t diagnose sleep disorders like sleep apnea. Most consumer devices do not sample at the resolution needed to detect and manage sleep disorders. I do believe that the technology will advance quickly, and consumer wearables will soon become just as good as medical-grade sleep tests.

Wearables are **extremely useful for trends**. They can show you whether your total sleep time is increasing, how consistent your schedule is, and whether HRV is improving. Think of them as a compass, not a GPS. They guide you, but they don’t tell the whole story.

I often get asked: “what’s the best sleep tracker”? My suggestion is to pick a form factor you like and can wear consistently, most nights. Some people prefer wrist-based ones, while others prefer rings. The value is in trends and not the absolute values.

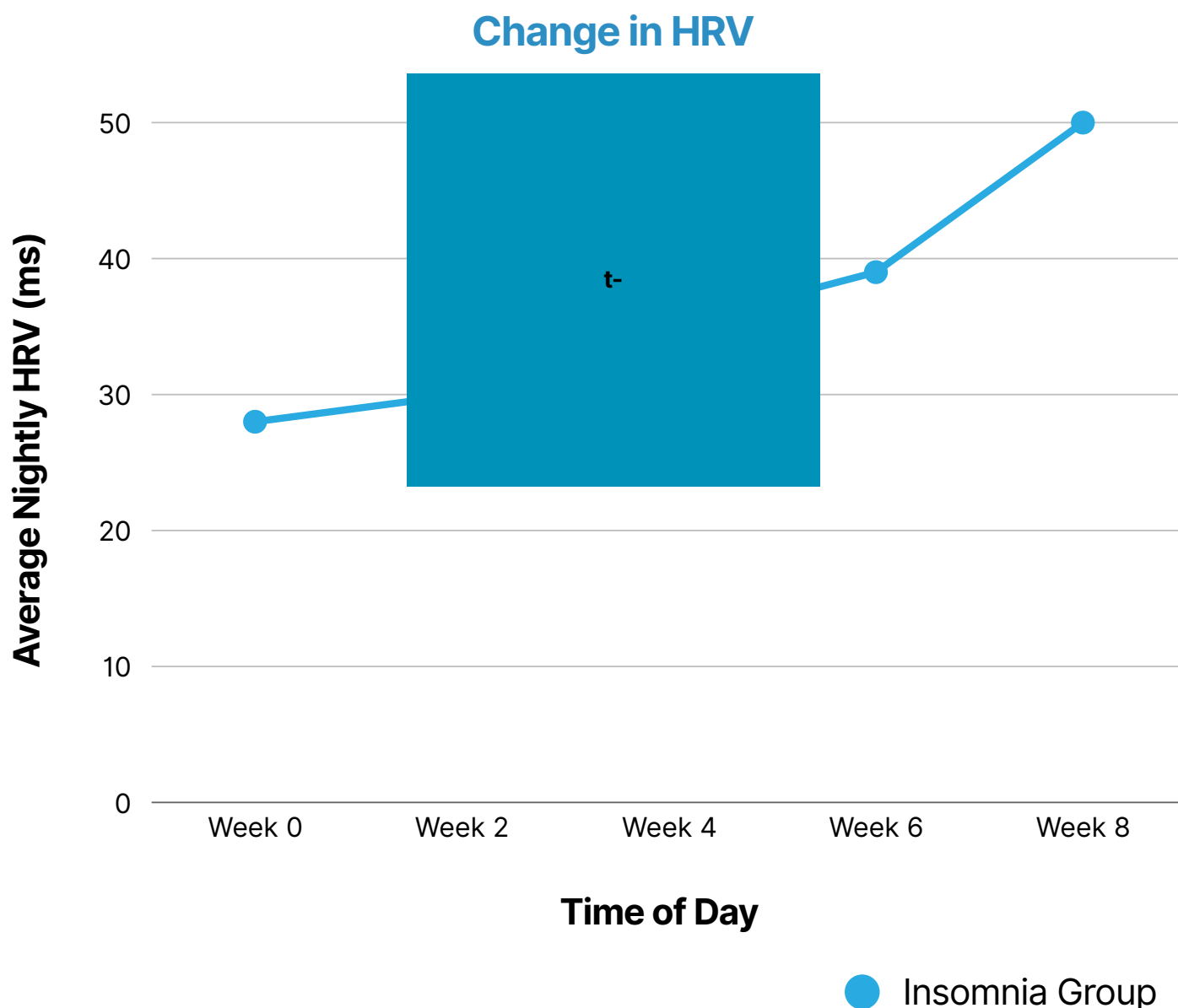


11

HRV AND INSOMNIA

A fascinating area of research links **Heart Rate Variability (HRV)** with sleep. HRV is a measure of how adaptable your nervous system is. Insomnia often lowers HRV, reflecting chronic stress and reduced parasympathetic activity.

Some of my patients track HRV with wearables. While it's not a diagnostic, it can show whether relaxation techniques and better sleep habits are helping restore balance over time.

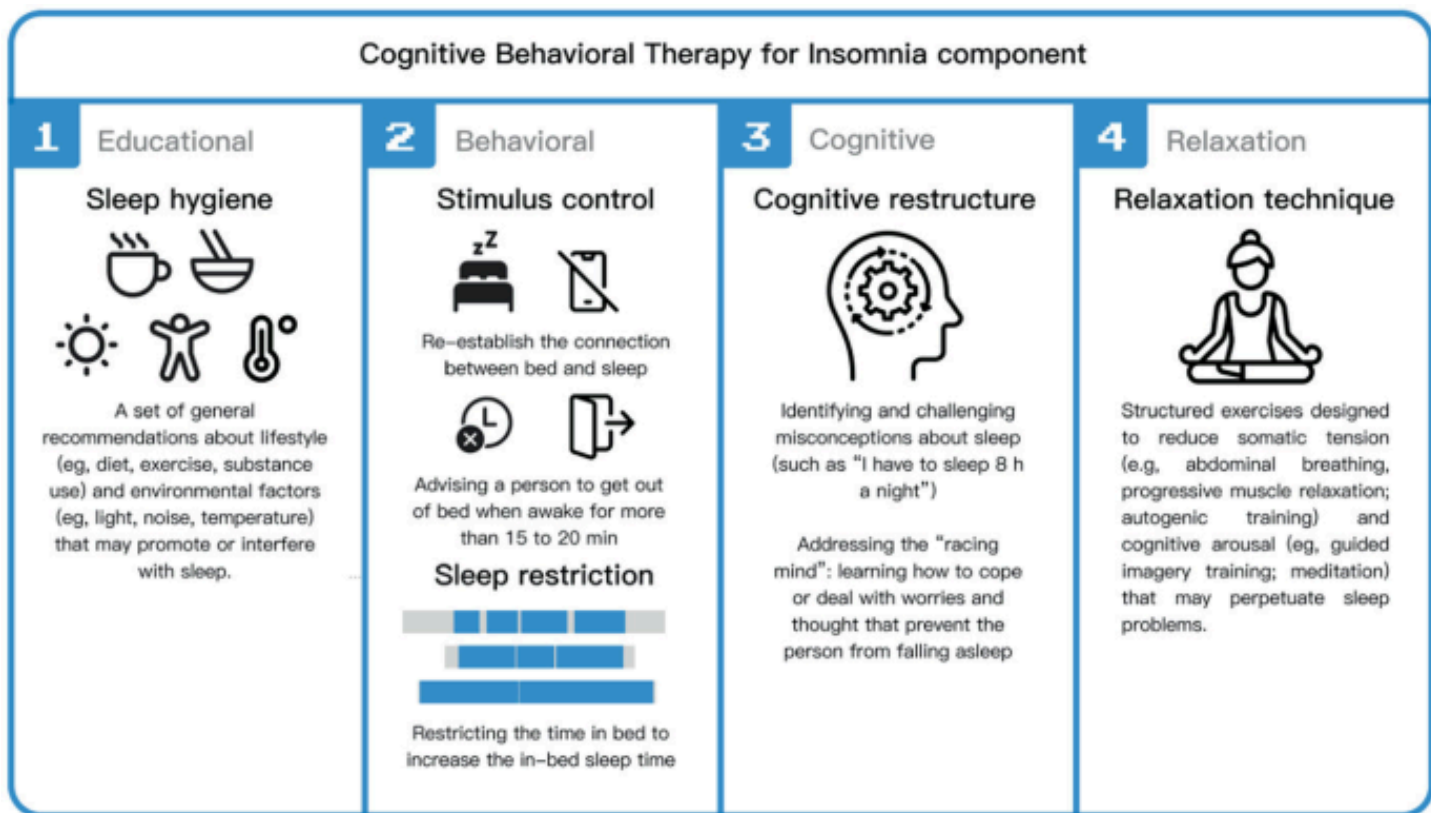


12 CBT-I EXPLAINED

Cognitive Behavioral Therapy for Insomnia (CBT-I) is the **gold standard treatment** for chronic insomnia. Unlike medications, which can often mask bad sleep behaviors, CBT-I retrains the brain’s sleep systems. It combines several core components:

- **Stimulus control** teaches your brain that the bed is for sleep, not for wakeful activities. For example, if you’re lying awake for more than 20 minutes, you get up, do something calming, and return only when drowsy
- **Sleep restriction** sounds counterintuitive. You actually limit your time in bed to match the amount you’re truly sleeping. Over time, this builds up sleep pressure, allowing you to fall asleep faster and stay asleep longer
- **Cognitive therapy** helps you challenge anxious thoughts like, “if I don’t sleep tonight, tomorrow will be a disaster.” Instead, we replace them with balanced thoughts, reducing the mental pressure that fuels insomnia
- **Relaxation training** includes tools like progressive muscle relaxation, deep breathing, or guided imagery, which calm the nervous system before bed
- **Sleep hygiene** is critical. It provides the environmental and lifestyle foundation such as keeping the room dark and cool, avoiding caffeine late in the day, and winding down before bed

What makes CBT-I unique is that these strategies work **synergistically**. On their own, any single tool may help a bit, but together they systematically retrain both body and mind to sleep more efficiently. The results are both durable and sustainable. Patients who complete CBT-I often maintain their improvements for years.



COMMON BARRIERS AND SOLUTIONS

I know change isn't easy. Some people say "I can't shut off my brain." For them, journaling or mindfulness can help. Others struggle with early morning awakenings, which can be improved by optimizing light exposure and room temperature.

If sleep restriction feels unbearable, I remind patients that it's temporary and can be adjusted gradually. And if a wearable reports frighteningly low numbers, I encourage focusing on function: "how do you feel?" matters more than the device's sleep score.

Barrier	Solution(s)
Difficulty falling asleep	Establish a consistent wake up time & bedtime routine; avoid screens before bed; encourage relaxation techniques (e.g. deep breathing, meditation, etc.)
Frequent awakenings during the night	Limit caffeine and alcohol, especially in the evening; ensure a dark, quiet, and cool sleep environment. Get out of bed if you've been awake for longer than 20-30 minutes
Feeling unrested despite enough sleep	Address potential underlying medical conditions (e.g., sleep apnea); improve sleep hygiene; consider a new mattress/pillow
Irregular sleep schedule	Go to bed and wake up at roughly the same time every day, even on weekends; avoid naps
Stress and anxiety keeping you awake	Practice stress-reduction techniques; engage in light exercise earlier in the day; consult a therapist if needed

14 SUPPLEMENTS

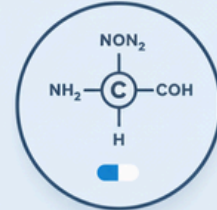
Sometimes, short-term use of sleep medications is appropriate. But they are not a long-term fix. Supplements like melatonin, magnesium, and glycine may help specific individuals. As with all interventions, personalization is key and it's best practiced under physician guidance. **These are not first-line and can be considered only if you've completed the basics.**



Melatonin



Magnesium



Glycine

15 MEDICATIONS (SEDATIVES AND HYPNOTICS)

While behavioral therapies like CBT-I are the gold standard for chronic insomnia, medications can play a role in select situations. I tell my patients that sleep medications are tools, not cures. They may help in the short-term to break a cycle of sleepless nights, but may not address the root cause of insomnia.

There are several classes of prescription hypnotics:

- **Benzodiazepines** (temazepam, clonazepam, alprazolam, lorazepam, etc.) act quickly but carry risks of dependence, tolerance, and cognitive impairment, especially in older adults.
- **Non-benzodiazepine “Z-drugs”** (like zolpidem and eszopiclone) often have fewer next-day side effects. But they can still impair memory, coordination, and lead to complex sleep behaviors (sleepwalking or doing things in your sleep that you don't remember the next day).
- **Melatonin receptor agonists** (such as ramelteon) mimic the body's own sleep hormone and are generally safe for long-term use. Their effects are modest.

- **Dual Orexin Receptor Antagonists (DORAs)** (like suvorexant, lemborexant, and daridorexant) are a newer class that work by quieting wake-promoting pathways in the brain. Studies suggest they may improve both sleep onset and maintenance, with relatively low risk of dependence.

When I prescribe medications, I use the lowest effective dose for the shortest period of time, and always combine them with CBT-I.

There should always be an exit plan and the goal is to eventually taper off medications while maintaining good sleep through behavioral strategies.



Benzodiazepines

- **Mechanism:** GABA receptor positive modulator
- **Onset:** Rapid
- **Risks:** Dependence, cognitive impairment, anterograde amnesia, respiratory depression



Non-benzodiazepine "Z-drugs"

- **Mechanism:** Selective GABA receptor modulator
- **Onset:** Rapid
- **Risks:** Complex sleep behaviors, dependence, anterograde amnesia



Melatonin receptor agonists

- **Mechanism:** MT1/MT2 receptor agonist
- **Onset:** Relatively rapid
- **Risks:** Mild side effects, minimal dependence risk



Dual Orexin Receptor Antagonists (DORAs)

- **Mechanism:** Orexin receptor antagonist
- **Onset:** Rapid
- **Risks:** Next-day somnolence, contraindicated in narcolepsy, low dependence risk

16

WHEN TO SEEK PROFESSIONAL HELP

If after eight weeks you're still struggling, or if your insomnia is severe and accompanied by depression, anxiety, or other medical issues, it's time to seek professional help. Insomnia is treatable, but sometimes it requires structured therapy or sleep studies to rule out conditions like sleep apnea.

Get Personalized Sleep Care, Guided By Experts

Our virtual sleep care program uses your data to create a plan that works for you

www.empowersleep.com

IF YOU HAVE ANY QUESTIONS, MESSAGE US ON OUR SOCIAL CHANNELS.
WE CAN MAKE A VIDEO TO ANSWER YOUR QUESTIONS.

   @empowersleep

You could also email us at care@empowersleep.com

17

CLOSING THOUGHTS

The journey through insomnia is rarely linear. Some weeks will feel like breakthroughs while others have setbacks. That's normal. What matters is persistence. By applying evidence-based strategies and treating sleep as a lifelong skill, you'll restore not just your nights, but the energy, focus, and vitality of your days.

A woman with dark hair is sleeping peacefully in a bed with blue bedding. The background is a deep blue night sky filled with stars and soft clouds. A warm, glowing lamp is visible on a bedside table to the right.

 empower sleep